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NATO PRINCIPLES AND POLICIES OF MEDICAL SUPPORT

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FOR THE MILITARY COMMITTEE:

for *Jo E. Rod*
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NOTE: This Final Decision Sheet shall now be attached to MC 0326/3 as the top sheet. Page numbering of the complete document when this decision is attached is as follows:

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NATO PRINCIPLES AND POLICIES OF

MEDICAL SUPPORT

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NATO PRINCIPLES AND POLICIES OF MEDICAL SUPPORT

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CHAPTER ONE - INTRODUCTION

1.1 The Role of Military Medical Support in a changing environment. The ultimate role of military medical support – supporting the troops in performing their tasks by preserving and restoring their health and fighting strength - remains unchanged. The changing global environment however poses new tasks upon the Alliance, which require agility and flexibility from Combat, Combat Support and Combat Service Support Units, including medical support. Whilst the Alliance still requires the ability to generate medical support for large scale conventional operations, this is no longer the primary focus as the context in which armed forces operate and their military medical support is provided has changed fundamentally. The current environment in which NATO forces operate pushes nations toward seeking a multinational approach to medical support tasks. These new challenges may include enhanced support to the host nation and other entities (non-governmental organizations - NGOs, other allies, etc.) working in the joint operational area. This is a significant challenge with regard to interoperability, but – when functioning properly – opens options for burden sharing, achieving economy of scale, modular capability development and efficient use of limited national resources. The intention of this Military Committee policy document is to provide comprehensive general principles and policies applicable both for national and for NATO operational purposes. Discrepancies between national and NATO principles and policies have been proven to lead to shifts in national priorities resulting in degradation of deployable and sustainable national medical capabilities, which prevents their operational delivery in support of the Alliance's Level of Ambition. Nations should adapt their concepts, structures, planning guides and procedures accordingly.

a. Changes in Society. Public expectations of high quality medical support continually increase. Employers have increasing legal responsibilities to protect their employees' health and, in many NATO nations, much of this legislation applies to the military, even on operations. The public opinion and political reactions are sensitive to media coverage of health and medical support to the troops.

An effective and reliable military medical support system helps to improve troop morale and maintain the trust of military personnel, the wider public, and its political leadership. In all nations the military wishes to be, and to be seen to be, an employer of the highest standards. Whilst accepting that the military can be a hazardous occupation, public opinion does not accept avoidable risks. Consequently, there is now a requirement for the health of the forces to be assigned a very high priority before, during and after any deployment.

b. Changes in Medicine. Medicine has become a highly specialised and technical field and military medicine has often led the way. Military medicine has broadened beyond the traditional clinical domains to include disciplines such as preventive medicine, medical intelligence, epidemiological surveillance and screenings, telemedicine, occupational and environmental health or forensic aspects.

c. Changes in the Military. The focus of NATO has changed to peacetime security and crisis response operations, undertaken from a home base that rarely feels threatened. Few NATO nations now depend on conscription and individual

military personnel are much more specialised. Forces are becoming smaller as fewer people produce more fighting power; consequently there is a larger loss of capability if an individual becomes unfit for duty. Regular forces have found it increasingly difficult to recruit and retain medical professionals, and medical equipment has been more difficult to fund from restricted budgets. For many nations, medical shortfalls have become a severe limitation upon their operational capability, thus also limiting the freedom of action for the Alliance as a whole. To effectively address the rising new challenges, the medical community constantly monitors the situation, seeking mechanisms to bring military medical capabilities into balance with force needs. These basic mechanisms include, but are not limited to, increasing effectiveness and efficiency of the existing medical capabilities, exploiting the benefits of multinational cooperation, and investments into development of new medical capabilities and additional capacities.

Management of medical shortfalls requires sustainable efforts and continuous attention of NATO and national political and military decision makers. NATO however can only provide the framework for effective cooperation, while the primary role and responsibility for shortfall management rests with the nations. Success depends on the nations, whether they are willing to accept and consequently adequately resource and implement the proposed solutions for shortfall management.

Consequently, multinational medical support options have become increasingly necessary and require more complex coordination at each staff level. Within the NATO command structure the medical function has significantly grown in relevance and has been allocated broader responsibilities, e.g. in the area of defence planning.¹ Integrity and adequacy of military medical support capabilities remain an important prerequisite of mission success.

d. Changes in the Global Environment. Changes in global climate, economy and technology pose new challenges and result in geo-political areas of interest which might become focal points for possible future NATO operations. Some predictable basic characteristics of future operations need to be considered for medical planning and capability development. These characteristics include, but are not limited to, the following features:

- Global engagement
- Extended lines of communication
- Difficult conditions (climatic, geographic, socio-cultural and epidemiologic)
- High operational tempo
- Extended sustainability
- No or very limited host nation support available
- Public opinion less supportive of long engagements.

e. Changes in Threat. Beside the conventional scenarios, NATO now faces the threat of asymmetric conflict and terrorism, with the civilian society, as well as the military, at risk of attack. Although in many countries, civil protection is primarily

¹ Military medical has been recognized as a planning domain and is introduced into the NATO Defence Planning Process to reflect the growing demand for, and exploit the opportunities rendered by a NATO wide coordinated capability planning and development effort in support of the Alliance's Level of Ambition.

the responsibility of civilian authorities, military medical support continues to develop unique and specialist capabilities in areas such as diagnosis and care of casualties arising from the use of Weapons of Mass Destruction (WMD), and may be called upon to assist civilian authorities.

1.2 The Effect of Changes upon NATO's Medical Support Planning. The influence of these factors leads to an increasing requirement for prioritized national defence planning with a shift towards development, delivery, maintenance and preservation of deployable medical capabilities in support of the Alliance's Level of Ambition while exploiting all benefits of multi-national medical solutions and adequate medical staff elements to provide them.

PURPOSE

1.3 This document outlines the general principles of medical support and presents NATO medical policies that are derived from them. This document seeks to identify the aspects of NATO's operational policies unique to medical support. As well as directing the conduct of NATO operational medical support, these policies are intended to guide nations in developing compatible medical support concepts, plans, structures and procedures. Though providing the framework for operational clinical practice, clinical practice is not within the scope of this document and technical medical terms have been kept to the minimum consistent with clarity.

1.4 Military Medical Doctrine. The disciplines of military and civilian medicine are one of the most closely aligned areas of civil-military interaction, particularly in the areas of medical law and clinical training. This relationship and the highly specialised aspects of military medicine lead to the requirement for specialised military medical doctrine. Medical support will operate within the framework of NATO combat service support structures. NATO medical doctrine at all levels will be derived from specific medical principles and policies given in this document.

APPLICABILITY

1.5 The fundamental principles and policies set out in this document apply to the full spectrum of NATO operations. The Strategic Commands are to translate the medical principles and policies into their doctrine, concepts, directives and procedures.

IMPLEMENTATION

1.6 Effective medical support to forces for NATO, or NATO-led, operations is fundamental to mission success and the provision of appropriate and visible medical support demonstrates military resolve. By the prevention of disease, the rapid and effective treatment of the injured, wounded, or diseased, and their medical evacuation and eventual recovery and return to duty, the medical services make a major contribution to force protection and sustainability. These principles and policies require implementation in national support concepts to enable comprehensive medical support to NATO operations.

1.7 This medical policy document does not stand alone and is linked to other NATO policy in a number of areas. It is complementary to other documents applicable to the wide range of NATO operational support functions. The key documents concerned are listed in Annex B.

CHAPTER TWO - PRINCIPLES

2.1 This Chapter contains the fundamental principles governing the provision of medical support by NATO.

HEALTH AS AN ASPECT OF MILITARY OPERATIONS

2.2 Health is a key force multiplier of fighting power; only a healthy force can function at, and sustain, maximum effort. Health is not merely the absence of injury or disease; in its widest sense it includes physical and mental well-being. In an operational context, health is the ability to carry out duties unimpeded by physical or psychological problems.

2.3 Responsibility for the Health of NATO Forces. At all times, nations remain the risk owners; therefore they retain their legal duty of care as an employer of their military. However, upon Transfer of Authority, the NATO commander shares that responsibility. Increasingly, due to national capability shortfalls, medical support is delivered via multinational solutions, gradually shifting responsibility more to the NATO commander².

Coupled with this is the requirement to ensure the ability of the Alliance to generate forces and their support capabilities for operations. Multinational solutions in this area are based on common risk and burden sharing, and on acceptance of the Alliance's defence planning targets by the nations. Acceptance of these capability requirements results in appropriate national medical capability development plans and resource allocation in support of the Alliance's Level of Ambition. The medical personnel will advise on health matters and nations deliver the medical care required, but only the Commander can balance the health and medical risks involved in his plan and decide if they are acceptable.

HEALTHCARE PRINCIPLES

2.4 Compliance with the Law of Armed Conflict and Humanitarian Conventions. The conduct of medical activities will comply with the rules and spirit laid down by the Law of Armed Conflict³. In circumstances where specific provisions of these Conventions may not be directly applicable, these principles nevertheless define the minimum acceptable

² The level of responsibility held by the NATO commander will be governed by the agreements made by the nations establishing the units concerned. Even when a Multinational Medical Unit (MMU) is not under NATO command, the NATO staff may exercise a co-ordinating and mediating function between the nations.

³ In this context, law of armed conflict includes the provisions of the Hague and Geneva Conventions that are in force, as well as other applicable conventions. The Geneva Conventions of 1949 are widely accepted as customary international law. Not all NATO Member States have accepted the Additional Protocols to the Geneva Conventions of 1977. However, a number of Articles in both Additional Protocols are considered to be customary international law binding on all nations regardless of ratification of Protocols I and II. NATO personnel must follow their respective national law in determining the applicable international law binding on their actions.

standard. Primarily, within the framework of established mission population at risk⁴, without discrimination, all sick, injured, shipwrecked or wounded shall be treated solely on the basis of their clinical needs and the availability of medical resources.

2.5 Medical Ethics and Legal Constraints. Whilst all military personnel are bound by military laws and regulations, medical personnel also have additional individual responsibilities to the ethical and national legal requirements of their own clinical profession. Coordinated national and NATO Standing Operating Procedures will outline the legal framework for a secure activity of medical personnel in operations in accordance with all applicable law, especially in multinational medical support settings.

2.6 Best Medical Practise. The standards of care available can have a permanent effect upon the outcome and the effects of poor quality can rarely be reversed later. Every effort should be made to ensure that medical care is based on internationally accepted best medical practice. Compliance with this principle has to be ensured by a quality assurance system in order to achieve continuous improvement in healthcare support on operations (CIHSO). A valuable tool in defining, distribution and implementation of promising (best) medical practice is the medical lessons learned process.

2.7 Patient Welfare. In addition to physical well being as described above, the general welfare of patients is an important element of their health. Medical care is an important, however not the only, element of maintaining health and welfare. Other services (such as J1, J3, J4) also have important responsibilities and contribute in this area. Particular issues will be communication with command staff, relatives, management of personal affairs, psychological support, and social and spiritual welfare.

OPERATIONAL PRINCIPLES

2.8 Timeliness of Treatment. Time is a fundamental factor in the effectiveness of medical care. The time taken until the receipt of appropriate medical intervention will affect the general outcome of medical care, including the risk of death, the speed of recovery and the level of residual disability. Therefore, the allocation of medical resources has to ensure the timely provision of medical care. Trained non-medical first responders, capable of providing emergency life saving procedures as soon as possible within ten minutes after injury, will help to improve treatment outcomes and save lives. Treatment and evacuation timelines in operations are determined by medical doctrine (see also 3.10). Their implementation needs to be ensured by clear command and control over Medical Evacuation (MedEvac) assets.

2.9 Continuity of Care. A casualty's recovery will depend on the continuance of appropriate care. Continuity of care means uninterrupted and appropriate medical attention and response to the needs of casualties throughout the chain of their medical treatment and evacuation.

2.10 Compatibility of the Medical Support System. Adequate medical support is a fundamental element of any force development/generation process and medical elements

⁴ This military interpretation/application of the general guideline assures mission oriented effective use of the available medical capabilities, and reflects the military restraints that no dedicated military assets are for purely civilian purposes.

need to be as well prepared, as suitably equipped and trained, and as readily available for deployment as the forces they support. The main elements of compatibility are: readiness, mobility, protection and training.

2.11 Multinationality. Multinational medical solutions have considerable potential to reduce the burden of medical capability provision upon individual nations. However, the existence of national differences such as varying clinical protocols, different languages and legal restrictions, renders cooperation complex. Joint multinational training with a high degree of reality will pay many dividends for NATO operations by improving medical competencies supporting operational requirements and enhancing the interoperability of material, personnel and procedures of contributing nations. Multinational solutions may be extended to certain medical capabilities contracted from civilian providers to support operations for which military capabilities are not available or in which their use is impractical.

CHAPTER THREE - GENERAL POLICIES

3.1 This Chapter outlines the policies derived from the principles outlined in Chapter Two. These policies will govern NATO's provision of operational medical support and shall guide national medical capability development, training and preparation of the medical forces of member nations and others that may undertake deployments under a NATO umbrella.

RESPONSIBILITY FOR THE HEALTH OF NATO FORCES

3.2 **The Responsibilities of Commanders.** Whilst shared with nations, the duty of care for all personnel within a command rests with the Operational Commander. This encompasses the full spectrum of health and medical issues (including clear and tailored mechanisms of activation and use of national MedEvac assets, Medical Treatment Facilities and the medical lessons learned process⁵). Consequently, commanders need to be advised by medical staff in order to ensure their decisions and the actions of their subordinates which may affect the health of personnel are properly informed. The medical staff needs to be present within the operational headquarters, in order to be fully aware of the operational situation and able to respond in a timely manner to ensure optimal health and treatment outcome. Direct access of the Medical Advisor to the Commander is essential to ensure timely intervention in all health and medical support matters that require the Commander's attention, decision or action.

APPLICABLE STANDARDS OF HEALTHCARE

3.3 **Standard of Medical Care.** The aim of military medical support on operations is to support the commander's intention through timely and effective health and medical services to the troops to achieve outcomes of medical care equating to best medical practice. The application of this principle must be guided by the principles embodied within the concepts of Continuous Improvement⁶ in Healthcare Support on Operations and Evidence Based Medicine.

⁵ A valuable tool in defining, distribution and implementation of promising (best) medical practice.

⁶ Continuous improvement is the process by which best practice is shared, and challenges acknowledged and reflected upon, in order to learn from experience and so optimize healthcare support on deployed operations.

3.4 As NATO increasingly depends on multinational medical support⁷, it is crucial that each participating nation does not take exception in general to the quality of the clinical services contributed by each ally's military medical units. In addition to other measures, this demands special emphasis on each participating nation's adaptation of universally accepted Evidence Based Medicine (EBM) principles and Evidence Based Clinical Practice Guidelines (EB-CPG).

3.5 The Primacy of Clinical Need. Clinical need is to be the principal factor governing the priority, timing and means of a patient's medical care and evacuation.

3.6 The Universal Provision of Acute Emergency Care. Although the Operational Commander has the authority to limit the availability of military medical support to third parties, acute emergency treatment of life threatening conditions must not normally be denied within the capability/capacity of the medical resources deployed.

3.7 Medical Confidentiality. The confidentiality of the care provider-patient relationship should be respected in the military as it is in civil life. Therefore, sensitive clinical information is not to be communicated to any individual or organisation that does not have a legitimate need-to-know.

OPERATIONAL MEDICAL SUPPORT

3.8 Medical Planning Parameters. Comprehensive medical plans, a high state of medical readiness, medical intelligence and an early and continuous medical input into the force generation process are essential to provide an adequate medical support capability, coherent and effective from the onset throughout all phases of any operation. The capacity of the support system directly depends on the population at risk as defined by the operational concept, and should be derived from the expected casualty load and the operational health and medical risk. Significant changes of eligibility for military medical support (e.g. allied forces, host nation forces, civilian population) are to be taken into account in the course of a deployment.

3.9 Force Health Protection (FHP). Preventive Medicine and access to Primary Health Care are fundamental aspects of maintaining the health of personnel and the sustainability of forces. Disease and non-battle injury (DNBI) will be a persistent health risk to personnel. The application of a comprehensive FHP policy is required before, during and after any operation. Many of the issues involved in this will be beyond the scope of sole medical responsibility. Effective force health protection requires and incorporates such main elements as medical intelligence, medical chemical, biological, radiological and nuclear (CBRN) defence, or an early warning system of near-real time health surveillance, capable of identifying the first signs of man-made or natural epidemics.

3.10 Treatment Timelines. The aim of operational medical support is to ensure that every casualty gets the right treatment in a timely manner and at an appropriate facility.

⁷ The aim is to get nations to develop, commit, deploy and sustain medical capabilities that are capable of supporting more than one ally.

- a. Timeliness of treatment is a fundamental principle of medical support. The time taken to provide appropriate medical care will affect the outcome for the casualty, especially those who are seriously injured and who must be treated as soon as practicably possible. The initial response at point of injury is crucial. For the most seriously injured, provision of bleeding and airway control must be achieved within **10 minutes** of wounding.
- b. Planning is essential to ensure sufficient personnel are positioned throughout the force to meet this requirement and that they are trained, competent and equipped to provide the enhanced first aid skills needed. These personnel must in turn be supported by both skilled medical aid and timely evacuation to ensure each casualty reaches the most appropriate medical treatment facility, within the given timelines and in the best condition possible.
- c. It is important that a continuum of care is provided with the necessary treatment and evacuation capabilities available throughout the chain to meet clinical requirements. In this way medical support will save lives, minimise the effect of injury and create the conditions for effective rehabilitation that returns personnel to active military duty. The timelines on which NATO and national medical support planning should be based for support to military operations and exercises are described within AJP 4.10.

3.11 Clinical Environment. Medical units are the primary area of quality health assurance system implementation to provide the best possible clinical environment for casualty care, which is compatible with their role and operational situation.

CHAPTER FOUR - THE ORGANISATION OF OPERATIONAL MEDICAL SUPPORT

4.1 This Chapter outlines NATO's policy on the organisation and structures for operational medical support. Operational medical support capabilities are directly connected to national capability development plans, their implementation and correlation with the Alliance's Level of Ambition in regard of political guidance, determination and apportionment of requirements, setting the targets, facilitation of the implementation and review of the results. Operational medical support requires development, delivery, sustainment and preservation of deployable, expeditionary national medical capabilities.

4.1.1 Medical Input to Operational Planning. Medical support planning is an integral part of the operational planning process from the outset and has to be performed in close co-operation with all other staff divisions. Commanders and their staff will have to consider the impact of a major incident, or casualties on the operational plan and how they are to be cared for, especially in Mass Casualty (MASCAL) situations.⁸

COMPONENTS OF DEPLOYED HEALTH CARE

⁸ This principle justifies the need for an integrated pre-deployment training of command and key medical personnel. Such an integrated pre-deployment training opportunity may for example be created and utilized on the base of NATO School, NATO Centre of Excellence for Military Medicine, and Joint Warfare Center training capabilities.

4.2 The capabilities required comprise the following main functional systems:

- a. Medical Command, Control and Communication;
- b. Medical Intelligence;
- c. Force Health Protection (FHP);
- d. Integrated Treatment and Evacuation (including capabilities to work in an CBRN environment or to receive casualties from CBRN affected areas) and
- e. Medical Logistics (see paragraphs 4.18 and 4.19).

COMMAND, CONTROL AND COMMUNICATION

4.3 Direct access of Medical Advisors to their NATO commanders and other key command staff elements is the prerequisite for effective medical support. The subordination of medical staff within a logistics organisation is a potential obstacle to the provision of dedicated medical advice to operational commanders and carries the risk of adverse casualty outcomes on operations. Medical personnel must be fully integrated into the staff and operational planning processes and appropriately represented on reconnaissance teams. The medical staff must be adequate in size, equipment, training and experience, with clear and tailored authority to undertake appropriate and timely actions, including medical planning.

4.4 The Medical Director is responsible for timely medical planning and co-ordination. A dedicated and structured command and control system is the essential foundation of an efficient medical support structure. This system, supported by a reliable and secure communications and information management system must be capable of planning, executing, controlling, supporting and auditing the full spectrum of medical support functions. The medical command system should seamlessly provide all resources required to support treatment, evacuation and flow of information from initial point of wounding, injury or sickness through evacuation to definitive treatment and final disposition.

4.5 Casualty Regulation. There are two main aspects to Casualty Regulation, the management of the flow of casualties, particularly at times of high flow, and the direction of individual patients through the system primarily according to their clinical need and medical resource availability. The Casualty Regulation system should be capable of providing timely and accurate tracking information throughout the chain of evacuation.

4.6 Communications and Medical Information Management. The efficient management of medical information, particularly regarding patients, is a vital element of competent medical support planning. It is essential that this information is standardised and distributed rapidly to all authorised⁹ personnel. Principal areas of medical concern will be:

- a. Passage of Information. Medical decision-making is dependent on the efficient processing and distribution of environmental, tactical, and casualty data. Such

⁹ Authorised to receive it.

information is fundamental for planning, disease and health surveillance and administration.

b. Patient Tracking and Regulation. Both patient tracking and regulation require up to date and accurate information about individual casualties and the availability of treatment and evacuation assets.

c. Patient Data. In a multinational environment medical documentation should be interoperable throughout the theatre of operations and in all national contingents. Medical data¹⁰ must move with the patient through the evacuation system to definitive care. Clinical records, including post-mortem reports and evidence related to deaths in theatre, must be accurately maintained for the use of official national or international audit and inquiries if required. In order to support the use of clinical data for the evidence-based improvement of medical support, an interoperable Trauma Registry System needs to be implemented by nations and the NATO Command Structure.

d. Telemedicine. Telemedicine might allow high-level multinational medical support with a smaller and more capable medical "footprint" in theatre. Telemedicine is a mechanism for using advanced medical and communication technologies which enables healthcare providers (and potentially patients themselves) to obtain consultation (opinion, recommendation, etc.) from another provider, usually a specialist (e.g., dermatologist, radiologist, infectious disease specialist, etc.). It complements (though it cannot replace) the traditional direct provider-to-provider referral and consultation process. Unlike traditional consultations, Teleconsultation is accomplished electronically in a system in which the "distant" or consulted provider does not meet with the patient face-to-face.

The effective use of Telemedicine will materially assist in our goal of providing home-nation quality of care to our deployed service-members in the modern context of reduced deployed medical force structure. During planning for medical support of operations the increased communications bandwidth necessary to support its use needs to be taken into account.

MEDICAL INTELLIGENCE

4.7 One of the essential requirements of medical support in the decision making process is the availability of reliable, timely, specific and applicable medical intelligence, from the initial planning stage throughout the operation as well as during and after deployment. Such intelligence will include assessment of hazards of operational concern such as infectious diseases, environmental and industrial health issues, other public health events, and CBRN threats, as well as assessment of host nation and opponent medical capabilities. Medical Intelligence will support the chain of command with qualified intelligence preparation of the battle space, giving input to the overall force protection concept.

FORCE HEALTH PROTECTION

¹⁰ It is expected that paper-based medical documentation will partially or completely be replaced in the future by digitalized medical information.

4.8 Force Health Protection (FHP) measures are an essential element of every operational plan. Elements of the FHP programme, such as vaccination, may in fact be the decisive factor on how quickly an operation can commence, and protective measures will frequently require implementation as soon as the decision is taken to commit to action. The plan will continue throughout the deployment and must extend well into the post-deployment period.

4.9 Restriction of Movement. Advising the Operational Commander on the requirements for any Restriction of Movement measures is a medical responsibility whenever there is a suspected or confirmed outbreak of a contagious disease or use of Weapons of Mass Destruction (WMD).

4.10 Protection against the use of WMD requires a comprehensive and integrated approach including vaccination, chemoprophylaxis, collective and personal protection. Medical personnel in WMD scenarios must have all the necessary tools, equipment and assets, and be trained appropriately to be able to conduct effective consequence management, including timely casualty collection, decontamination, evacuation and treatment.

TREATMENT

4.11 Treatment Capabilities¹¹. To support force needs, treatment capabilities are to be designed, developed, delivered and sustained in a way to cover the whole range and entire scale of medical support requirements, from effective prevention, surveillance and evacuation, through seamless, timely and progressive treatment to rehabilitation. Alongside timely and efficient preventive measures, effective first aid, timeliness, continuity and progressiveness of care, advanced diagnostics and treatment equipment, aggressive intensive care and highly specialized medical personnel, supported by robust medical evacuation, effective command and control, and efficient medical information and communication systems are prerequisites of successful treatment outcome.

4.12 Structural Requirements. Treatment capabilities have to include both deployable (expeditionary) and stationary (out-of-theatre) elements, thus contributing to an integrated system of medical support. The aim is to enable the armed forces to reach their objectives both on deployments and at home, through preserving, maintaining and restoring the health and fighting strength of soldiers and the whole authorized population at risk, and by a comprehensive mental, physical and social rehabilitation of casualties where their return to duty is not achievable. While the out-of-theatre elements are almost entirely a national responsibility, deployable assets are frequently operated within the framework of multinationality. The structure and type of required deployable Medical Treatment Facilities (MTFs) will vary depending on the type of operation, the medical mission and environmental factors. These capabilities need adequate mobility, protection, operational and maintenance support, must be mission tailored.

4.13 Treatment Philosophy for Mass Casualty Situations. MASCAL management is a command responsibility. Medical is part of MASCAL management. There is a demand for maintenance of normal standards of individual medical care even on the battlefield.

¹¹ The role system will remain the focus of AJP level doctrine document.

However, it is important to ensure that the provision of time consuming care to a few is not to the detriment of the majority. Therefore, in operational situations where large numbers of casualties are being sustained, a shift in approach is required to ensure that the best quality of care possible is given to all.

MEDICAL EVACUATION

4.14 The task of transferring casualties during military operations is influenced by factors such as the operational environment, the weather, and the length and quality of evacuation routes. The successful management of evacuation tasks along with other necessities requires the timely availability of suitable medical evacuation assets in sufficient numbers. Medical evacuation is not merely the movement of patients under medical supervision to MTFs as an integral part of the treatment continuum, but also and mandatorily medical assistance to patients provided as necessary by medical personnel during evacuation. The three categories of medical evacuation, which apply to sea, land and air operations are:

- a. forward (from point of wounding to first treatment point);
- b. tactical (within theatre);
- c. strategic (out-of-theatre) evacuation.

4.15 The evacuation concept is (closely) inter-related with the medical footprint and the casualty rate and theatre holding policy. The robustness of the evacuation chain is directly related to and depends on the quantity and capability of the treatment assets that will be required in theatre. This fundamental principle provides the argument for establishing a robust medical evacuation system. A robust medical evacuation system requires the following qualities:

- a. Timeliness (see 2.8).
- b. Availability. The aim of the medical evacuation system is to evacuate casualties 24 hours a day, in all weather and sea-states, over all terrain and in any operational scenario. Operations of special forces or at sea may require specific solutions.
- c. Continuity (see 2.9).
- d. Multinational Interoperability (see 2.11).

4.16 Responsibilities. An effective medical evacuation system is a command responsibility. In support of the commander, the Force Medical Staff will coordinate such activity with the operations and movement staff in theatre. The provision of resources will be coordinated by medical planning staff and may comprise assets from a number of sources, including common use theatre assets, nationally owned assets, Host Nation Support and Third Party contracted¹² capabilities. Harmonization of the medical evacuation doctrines and capabilities is the responsibility of MEDEVAC asset contributors. Only properly co-ordinated procedures can assure the smooth transfer of patients within a multinational medical support structure.

4.17 Aeromedical Evacuation. Aeromedical assets are an increasingly essential element to ensure timeliness of medical evacuation. A centrally co-ordinated multinational

¹² Must not be considered as an option for forward medical evacuation.

aeromedical evacuation system (within and out of theatre¹³) offers the opportunity to enhance the efficiency of medical support.

MEDICAL LOGISTICS

4.18 General. The medical logistics system needs to ensure the sustainability of the medical support system under all operational conditions. National responsibility over planning and executing an effective medical logistics system remains the guiding principle for operational support; however the NATO Commander may exercise his authority to assure best possible coordination of national activities in this area. Economy of scale may result from coordinated supply to commonly funded multinational assets. The unique characteristics of medical materiel set it apart from other commodities, for the following reasons in particular:

- a. Protected Status. Medical supplies are protected under the terms of the Geneva Conventions, when properly marked and separately stored and distributed from combat supplies.
- b. Regulatory Aspects. The accounting, administration and use of medical supplies, and in particular controlled drugs, are governed by national and international regulations. The consumption and controlled disposal of medical materiel must be recorded for legal, environmental and asset control reasons.
- c. Handling Requirements. Tight controls and specialised management are required for medical supplies due to the technical and perishable nature of the materiel, especially, its often-limited shelf life and its sensitivity to storage, transport and environmental conditions.
- d. Importance. Seemingly insignificant items can have genuine life and death importance. There is a complex inter-dependence between treatment capability and the availability of medical materiel; the medical logistics system must contain the knowledge and responsiveness to meet short notice clinical demands.

4.19 Blood Products and Medical Gases. Blood products and medical gases require particular consideration (both in terms of generation and distribution) when planning medical logistics. Neither group is normally available via host nation support (HNS) and require extensive management in the transport chain.

SPECIALIST AREAS

4.20 Dental Care. Dental care is an integral element of operational medical support. Prevention and correction of dental disease will ensure a higher availability of forces to deploy and reduce the number who will subsequently need to be removed from theatre for such problems. Pre-deployment dental care and dental fitness¹⁴ are a national responsibility and must be a priority for forces held at high readiness for NATO deployments. Deployed oral surgery is important to help maintain force availability and to deal with dental trauma.

¹³ Tactical, operational and strategic evacuation.

¹⁴ See STANAG 2466

4.21 Psychological Support and Mental Health Care. Mental health casualties may include stress disorders and a range of other anxiety and depressive disorders, some of which will be complicated by physical injury. The aim is not to “medicalise” the management of these cases but, where possible, to rehabilitate individuals within their own units. If correctly managed, many can be returned to duty quickly. Mental health briefings are an element of FHP and should be utilised, as appropriate, before, during and after an operation.

4.22 Veterinary Support. The provision of animal health care, whilst sharing many of the same principles, is not subject to the policies in this document. However, members of the veterinary services may also perform functions of general medical support, such as in the areas of medical intelligence and public health.

CHAPTER FIVE - APPLICABILITY TO SPECIFIC MILITARY SITUATIONS

5.1 Governance, Reconstruction and Development. These principles are reflected as military strategic objectives in line with the unity of effort approach, though the overall responsibility for governance, reconstruction and development is shared with other actors than NATO.

a. NATO forces operate in a complex security environment. Mission objectives might require the military and its medical service personnel to contribute to stabilization of the Host Nation not only by provision of security. Reconstruction and development of the local infrastructure and basic community services including basic health services for the local population, and military medical support capabilities of the Host Nation Armed Forces may be a necessary precondition to achieve the mission end state¹⁵.

b. Reconstruction and Development activities are part of the overall NATO operational concept and will be performed in close coordination and/or cooperation with non-NATO agencies in balance with the available resources. Unity of effort therefore will need to focus not only on the local population, local government and the Allied Forces, but will have to take into consideration also the motives and objectives of external players and the opposing forces/malign actors. NATO needs to be prepared to engage in military medical support to stabilization and reconstruction activities in all phases of a conflict. Key Host Nation, civilian, international governmental and non-governmental partners need to be identified, and the framework of (military medical) cooperation needs to be outlined well in advance of an operation. A framework agreement with key partners will assure coordination and understanding in each other's motives, objectives, capabilities and limitations.

¹⁵ Detailed military medical strategy, tasks and responsibilities are covered in AJMedP-6; Allied Joint Civil-Military Medical Interface Doctrine in ACO Directive 83-1 Medical Support to Operations; and in Allied Command Operations – ACO Directive 83-2, Guidance for NATO Military Medical Services Involvement with Humanitarian Assistance and Support to Governance, Reconstruction and Development.

- c. These activities primarily take place in disaster relief and other non-Article 5 operations.

5.2 Special Operation Forces (SOF) medical support principles

- a. NATO SOF often operate in areas outside of the Joint Operational Area (JOA) and outside the range or capabilities of Aeromedical Evacuation platforms. SOF missions are characterized by the deployment of small independently operating units. These operations may require a high degree of independence from other deployed forces, limiting the options for medical support. Therefore SOF should possess and maintain medical skills to address SOF specific mission sets. SOF Medical Support personnel should have habitual relationships with SOF operational units and an understanding of SOF specific operational environment variables.
- b. Integration of Medical Planners during all phases of the mission planning cycle and during deployment (with operational mission understanding) is imperative. Medical support to SOF must be nested with medical planning at the joint force medical staff level. Provision of medical support beyond SOF capabilities depends on the thoroughness of advanced planning so that the conventional medical support structure umbrella is extended to cover the limited organic capability or to meet requirements for additional medical assets¹⁶.
- c. NATO SOF may be exposed to health risks not normally seen in the other areas of the operational theatre. It is imperative that relevant medical threat assessments be conducted and MEDINT, as well as medical recommendations for FHP, in addition to conventional requirements, take into account SOF-specific requirements.

5.3 INTERNATIONAL DISASTER RELIEF OPERATIONS¹⁷

5.3.1 Aim. Provide general medical policy guidance on NATO military support to International Disaster Relief Operations (IDROs).

5.3.2 General. The term “disaster relief” is used here to include humanitarian assistance, refugee care, and similar missions not directly related to combat or peacekeeping. IDROs can either be carried out within the framework of another ongoing operation or as a stand-alone humanitarian operation. In the latter case, the operation often involves a considerable degree of logistic and military protection or support, including medical support. The major contribution of medical support to such an operation is threefold: medical support to the NATO force, replacing or supplementing the existing Host Nation assets and assisting with their regeneration or development.

¹⁶ These additional assets may include, but are not limited to critical care evacuation teams, far forward damage control surgery, dental care, veterinarian support, and psychiatric care.

¹⁷ Comprehensive NATO guidance exists in relation to disaster relief operations. MC 327 provides policy on the planning and conduct of non-Article 5 Crisis Response Operations. MC 343 outlines the principles of military assistance in humanitarian emergencies not connected to any NATO military operation. MC 411 addresses civil-military interfaces, including military support for humanitarian emergencies within the context of other operations when NATO forces are already deployed or to be deployed.

5.3.3 Coordination of NATO-wide planning and consequence management efforts rests primarily with the Civil Emergency Planning Committee (CEPC), and the civil expertise to cover its medical aspects is cumulated within the framework of the Public Health Food Water Group (PHFWG), the former Joint Medical Committee, under professional medical control of COMEDS.

5.3.4 Differences from Combat Operations. The medical aspects of IDROs are likely to differ from other military deployments, particularly in types of injury and illness, the population mix and the structure and type of medical response required. The particular nature of the operation concerned and the geographical location will dictate the medical skill mix required. Clear guidance on legal issues such as legal immunity of medical personnel needs to be obtained prior to deployment.

5.3.5 Basic Tenets. In considering the deployment of military medical assets in Humanitarian Assistance, and a clear understanding of the philosophy behind such assistance is required in order that the affected population may be best served, both in the short and longer term. The following basic tenets must be observed:

- a. The affected population should be encouraged to help themselves and competition between their own medical structure and NATO's must be avoided;
- b. A return to the pre-crisis status quo, in terms of self-help, must be as rapid as possible to avoid long-term aid dependence;
- c. Mitigation of the disaster affects requires immediate medical assistance and the application of preventive measures; and
- d. Other (than NATO) organisations are likely to be involved. Care needs to be taken to define areas of responsibility, to avoid competition and either duplication of effort or gaps in the overall care provided. In principle, Medical Humanitarian Assistance will only be provided by NATO when Governmental or Non Governmental Organisations are unable to meet the overall requirement.

5.3.5. Standards of Medical Care. Medical support must meet standards acceptable to both the participating nations and the receiving country. The fundamental principle is that when military medical services are engaged in Medical Outreach activity this should primarily address health needs, be appropriate to the level of care that can be realistically and competently sustained within the Host Nation, and support the Host Nation health development programme.¹⁸

¹⁸ ACO Directive 83-2, Allied Command Operations Guidance for Military Medical Services Involvement with Humanitarian Assistance and Support to Governance, Reconstruction and Development

LEXICON

This Lexicon is provided to help with the understanding of this MC document. The official document for NATO terminology is the NATO Terminology Database. Other reference documents are the NATO Glossary of Terms and Definitions (AAP-6) and the NATO Glossary of Medical Terms and Definitions (AMedP-13).

continuity of care

A medical support principle which states that a patient both in-transit and passing through the various medical echelons must be given care which is relevant, continuous and progressive irrespective of the organization providing the resources
(AMedP-13)(A) RD1

continuous improvement in healthcare support on operations

A system through which medical organisations are accountable for continuously improving the quality of their services and safe guarding high standards of care, by creating an environment in which clinical excellence will flourish. The four basic components are a coherent approach to improving clinical effectiveness, continuing utilisation of audit, ongoing professional development and establishing effective processes for identifying and managing risk and addressing poor performance.

damage control surgery

DCS

Emergency surgical procedures and treatment to stabilize casualties, in order to save life, limb or function, including rapid initial control of haemorrhage and contamination, temporary closure, and resuscitation.

Note: These procedures should be followed later by primary surgery. Damage Control Surgery is a procedure that ensures surgical treatment at an early stage. Providing life, limb and function-saving surgery is different from primary surgery, which focuses on damage repair.

Related term: primary surgery

(AMedP-13)(A) RD1

emergency medicine

The clinical discipline specializing in the triage and immediate care of casualties in an acute care situation.

(AMedP-13)(A) RD1

evidence-based medicine

An approach to health care that promotes the collection, interpretation, and integration of valid, important and applicable patient-reported, clinical-observed, and research-derived evidence. The best available evidence, moderated by patient circumstances and preferences, is applied to improve the quality of clinical judgements.

(AMedP-13)(A) RD1

force health protection

Actions taken to counter the effects of the environment, occupational health risks, and

disease through preventive and reactive measures.
(AMedP-13)(A) RD1

force protection

All measures and means to minimize the vulnerability of personnel, facilities, equipment and operations to any threat and in all situations, to preserve freedom of action and the operational effectiveness of the force.

NATO Agreed

holding policy

a command decision for planning purposes indicating the maximum length of time (days) that a patient will be allowed to remain in the theatre for treatment, recovery and return to duty.

Explanation: If the prognosis is that recovery will take longer than the timelines set by the holding policy, then the patient should be evacuated as soon as he/she is considered suitable for evacuation.

(AMedP-13)(A) RD1

host nation medical support

Civil or military medical assistance rendered by a nation to foreign forces within its territory based upon agreements mutually concluded between nations or between a NATO command and a nation.

(MCMedSB)

medical advisor

MEDAD

The senior medical staff officer (physician) in a formation headquarters responsible for ensuring that the commander and his staff are properly aware of the health and medical implications of their actions and any force health issues connected to the operation.

Notes:

1. The Medical Advisor may also be designated as the Force or Theatre Medical Director.
2. This definition will require changes in AD 85-8.

Related terms: medical director

(AMedP-13)(A) RD1

medical director

MEDDIR

The functional head of the medical services in a formation or theatre of operations.

Note: The Medical Director may also have the additional responsibilities of being the Medical Advisor to a senior commander

Related terms: medical advisor

(AMedP-13)(B)SD9

medical estimate

The results of assessing the essential elements of the situation, identifying any shortfalls, leading to the identification of required medical capabilities as a prerequisite for the development of medical courses of action.

(MC Medical Standardisation Board)

medical evacuation

The medically supervised process of moving any person who is wounded, injured or ill to and/or between medical treatment facilities as an integral part of the treatment continuum.

Related terms: patient transfer

(AMedP-13)(A) RD1

medical intelligence

MedIntel

Medical Intelligence / Renseignement médical is intelligence derived from medical, bio-scientific, epidemiological, environmental and other information related to human or animal health.

Note: this intelligence, being of a specific technical nature, requires medical expertise throughout its direction and processing within the intelligence cycle.

(AJMedP-3)

medical logistics

The process of procurement, storage, movement, distribution, maintenance and disposition of medical materiel including blood, blood components, and medical gasses, in order to provide effective medical support. The application of this process in planning and implementation.

(AMedP-13)(A) RD1

medical planning

The process of designing, requiring and acquiring medical support capabilities in balance with the mission, force structure and risk exposure, and in accordance with the politico-military requirements.

(AMedP-13)(A) RD1

medical regulating

The process of directing, controlling and coordinating the transfer of patients, from point of wounding or onset of disease through successive MTFs, in order to facilitate the most effective use of medical treatment and evacuation resources and to ensure that the patient receives appropriate care in a timely manner.

(AMedP-13)(A) RD1

medical service

A branch of a military or civilian organization responsible for establishing medical policy and plans, and for the provision of medical support.

(AMedP-13)(A) RD1

medical services

Activities related to all professional, technical, and related functions performed by physicians and/or other health care provided under the direction of a physician or another medical professional. Those organizational branches of the military that provide those professional, technical, and related functions performed by military medical personnel.

(AMedP-13)(A) RD1

medical support

The function encompassing the full range of medical planning and provision of medical and health services to maintain the force strength through disease prevention, evacuation, rapid treatment of the diseased, injured, and wounded, their recovery and return to duty.

(AMedP-13)(A) RD1

mobility

A quality or capability of military forces which permits them to move from place to place while retaining the ability to fulfill their primary mission.

NATO Agreed

operational readiness

See: readiness, operational

operational training

See: training, operational

patient transfer

The evacuation of casualties or patients from one medical treatment facility to another, under clinical supervision and utilising specialised medical transport,.

Related terms: medical evacuation.

(AMedP-13)(A) RD1

population at risk

A group of individuals exposed to conditions which may cause injury or illness.

(AMedP-13)(A) RD1

preventive medicine

The services that are concerned with identifying, preventing, and controlling acute and chronic communicable and non-communicable diseases and illnesses, with food and environmental hygiene, and with vector control.

(AMedP-13)(A) RD1

primary health care

The provision of integrated, accessible health care services by clinical personnel trained for comprehensive first contact and the continuing care of individuals experiencing signs and symptoms of ill health or having health concerns.

Explanation: Primary Health Care includes health promotion, disease prevention, patient education and counselling, and the diagnosis and treatment of acute and chronic illness.

(AMedP-13)(A) RD1

primary surgery

The surgical procedures directed at repair of the local damage caused by wounding, rather than correcting the generalised effects, which is performed normally at Role 3.

Note: Delays to primary surgery allow further generalised effects to develop that may lead to an increase in mortality, morbidity and residual disability.

Related term: damage control surgery

(AMedP-13)(A) RD1

protection

An arrangement for restricting access to or use of all, or part, of a computer system.
(ISO) the International Organization for Standardization
NATO Agreed

protection, force

See: force protection

protection, force health

See: force health protection

readiness

Note: in English, "readiness" is a generic term covering "readiness state" and "readiness time".

Related term(s): operational readiness. readiness state. readiness time.

NATO Agreed

readiness, operational

The capability of a unit/formation, ship, weapon system or equipment to perform the missions or functions for which it is organized or designed. May be used in a general sense or to express a level or degree of readiness.

NATO Agreed

readiness state

The measure of the capability of forces at a given point in time to execute their assigned missions.

NATO Agreed

readiness time

The time within which a unit can be made ready to perform the tasks for which it has been organized, equipped and trained. This time is amplified or measured by indicators of the unit's current personnel, materiel and training state. It does not include transit time.

NATO Agreed

secondary health care

The provision of hospitalisation and specialised clinical care requiring training and equipment levels beyond that which could normally be provided at the level of primary care.

Explanation: Routine access to these services will normally be by referral from Primary Health Care. Urgent access will normally be via an Emergency Medicine department. This capability will be found, in varying degrees of capability, at Roles 2 to 4 Medical Treatment Facilities.

(AMedP-13)(A) RD1

telemedicine

The use of information and communications technologies to exchange health information and provide healthcare services regardless of time and distance.

Note: depending on the clinical specialty involved, this may incorporate such terms as teleradiology, teledermatology, telesurgery, telepathology and telepsychiatry.

(AMedP-13)(A) RD1

training

The process of learning a skill or type of behaviour through regular practice and instruction.

(COED) Concise Oxford English Dictionary

training, operational

Training that develops, maintains or improves the operational readiness of individuals or units.

NATO Agreed

trauma registry

The electronic collection of selected clinical and non-clinical data from trauma patient records in a standardised format that can be statistically analysed for performance improvement of clinical care.

(MCMedSB)

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